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Dataset note: This case section was extracted mechanically from the cited PMC article. No clinical facts were added.

2. Case Presentation

An 87-year-old male with medical history of paroxysmal atrial fibrillation on 20 mg of rivaroxaban daily, recent pulseless ventricular tachycardia with implantable cardiac defibrillator in situ, and nonischemic cardiomyopathy (ejection fraction 35%) presented to the emergency room (ER) with intermittent chest pain and light headedness of 2 days duration. Chest pain was exertional, left sided, and pleuritic. He also reported shortness of breath but denied cough, fever, or any other infectious. Of note, he was commenced on 200 mg amiodarone daily four months prior to presentation following an episode of syncope due to pulseless ventricular tachycardia.

Initial vital signs showed blood pressure of 89/60 mmHg with pulse rate of 59/min, temperature of 98.2°F, and respiratory rate of 14 breaths/min with normal oxygen saturation of 100% on ambient air. On physical examination, he was in no acute distress and was alert and oriented to time, place, and person. Jugular venous distention was noted on neck examination. Heart sounds were muffled, but lung fields were clear to auscultation. Peripheral pulses were also weak but palpable.